CRISIS & CRISIS INTERVENTIONS

Objectives:

At the end of this lecture, students should be able to:

1. Define crisis.
2. Identify characteristics of a crisis.
3. Explain phases of the development of a crisis.
4. Understand types of crises.
5. Discuss techniques of crisis intervention.
6. Describe modalities of crisis intervention.

Introduction:

Stressful situations are a part of everyday life. Any stressful situation can precipitate a crisis. Crises result in a disequilibrium from which many individuals require assistance to recover.

Crisis intervention requires problem-solving skills that are often diminished by the level of anxiety accompanying disequilibrium.

Assistance with problem-solving during the crisis period preserves self-esteem and promotes growth with resolution.

Crisis Theory Origin

Erikson (1950-1959)

- Normal personality growth occurs through resolution of normative developmental crises
- Those who resolve problems in previous stage achieve lasting solution, new skill, confidence etc.
- Potential for development or vulnerability.

Caplan (1961)

- Unscheduled crises or life events, when successfully resolved, also promote growth.

CHARACTERISTICS OF A CRISIS:

The concept of crisis is based on (Aguilera, 1998; Caplan, 1964; Kaplan & Sadock, 1998) includes the following:

- Crisis occurs in all individuals at one time or another and is not necessarily equated with psychopathology.
- Crises are precipitated by specific identifiable events.
- Crises are personal by nature. What may be considered a crisis situation by one individual may not be so for another.
- Crises are acute, not chronic, and will be resolved in one way or another within a brief period.
- A crisis - situation contains the potential for psychological growth or deterioration.

DEFINITIONS OF CRISIS:

A sudden event in one's life that disturbs homeostasis, during which usual coping mechanisms cannot resolve the problem.

PHASES IN THE DEVELOPMENT OF A CRISIS:

Caplan (1964) outlined four specific phases through which individuals' progress in response to a precipitating stressor.

Phase 1. The individual is exposed to a precipitating stressor.

Anxiety increases; previous problem-solving techniques are employed.

Phase 2. When previous problem-solving techniques do not relieve the stressor.

Anxiety increases further. The individual begins to feel a great deal of discomfort at this point. Coping techniques that have worked in the past are attempted, only to create feelings of helplessness when they are not successful. Feelings of confusion and disorganization occurred.

Phase 3. All possible resources, both internal and external, are called on to resolve the problem and relieve the discomfort.

The individual may try to view the problem from a different perspective. New problem-solving techniques may be used, and, if effectual, resolution may occur at this phase, with the individual returning to the previous level of premorbid functioning.

Phase 4. If resolution does not occur in previous phases, Caplan states that "the tension increases over time to a breaking point. Major disorganization

of the individual often occurs." Anxiety may reach panic levels. Cognitive functions are disordered, emotions are labile, and behavior may reflect the presence of psychotic thinking.

Balancing factors:

The previous phases are congruent with the concept of "balancing factors" as described by Aguilera (1998).

These factors affect the way in which an individual perceives and responds to a precipitating stressor.

A crisis depends on the following three factors:

1. **The individual's perception of the event.**

If the event is perceived realistically, the individual is more likely to draw on adequate resources to restore equilibrium.

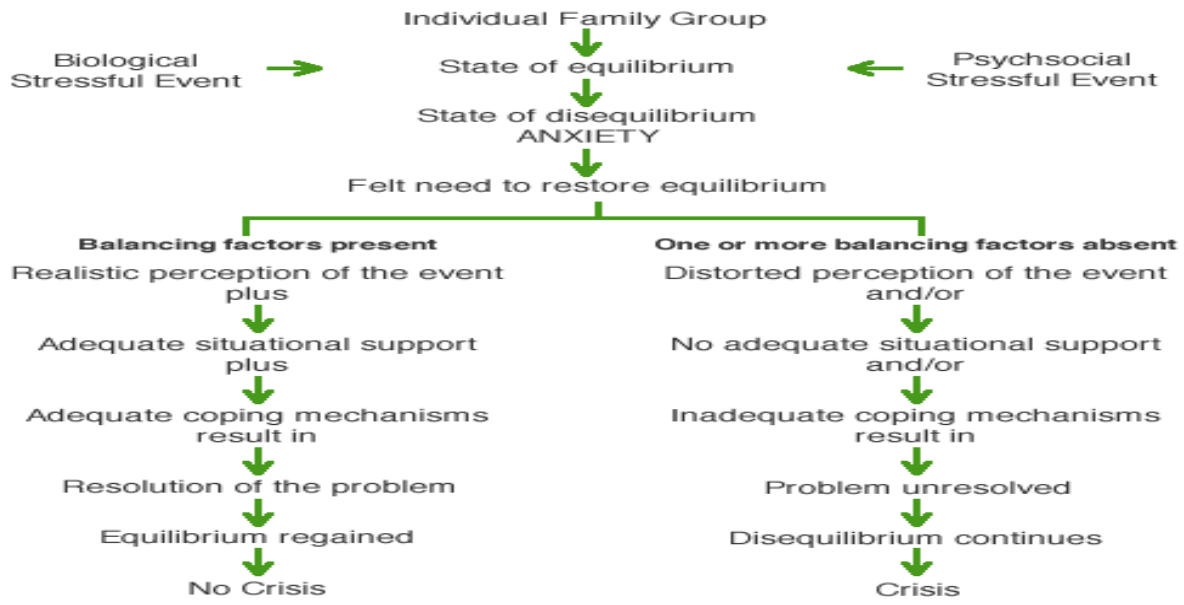
If the perception of the event is distorted, attempts at problem-solving are likely to be ineffective and restoration of equilibrium goes unresolved.

2. **The availability of situational supports.** Aguilera states, "Situational supports are those persons who are available in the environment and who can be depended on to help solve the problem."

Without adequate situational supports during a stressful situation, an individual is most likely to feel overwhelmed and alone.

3. **The availability of adequate coping mechanisms.**

When a stressful situation occurs, individuals draw on behavioral strategies that have been successful for them in the past. If these coping strategies work, a crisis may be diverted. If not, disequilibrium may continue and tension and anxiety may increase.



Aguilera, 1998 (Crisis model)

TYPES OF CRISES

1) Maturational crisis:

It is a stage in a person's life where adjustment and adaptation to new responsibilities and life patterns are necessary.

The transition points where individuals move into successive stage often generate disequilibrium. Individuals are required to make cognitive and behavioral changes and to integrate those physical changes that accompany development.

Ex., (adolescence, marriage, parenthood, midlife and retirement)

2) Situational crisis:

It is one that is precipitated by an unanticipated stressful event that creates disequilibrium by threatening one's sense of biological, social or psychological integrity.

Ex., (premature birth, death of loved one, divorce, physical or mental illness, change geographical location& poor performance in school)

3) Social crisis:

It is accidental, uncommon, and unanticipated and results in multiple losses radical environmental changes.

Ex. (natural disasters like floods, earthquakes, contamination of large areas by toxic wastes, wars, etc.)

THE CONSEQUENCE OF UNRESOLVED CRISIS→SUICIDE!!!!

Definition of Crisis Intervention:
Crisis intervention is a brief, focused, and time-limited treatment strategy that is effective in helping people adaptively cope with stressful events.

Techniques of Crisis Intervention:

Technique: Catharsis
Definition: The release of feelings that takes place as the patient talks about emotionally charged areas.
Example: "Tell me about how you have been feeling since you lost your job."

Technique: Clarification
Definition: Encouraging the patient to express more clearly the relationship among certain events.
Example: "I've noticed that after you have an argument with your husband you become sick and can't leave your bed."

Technique: Suggestion
Definition: Influencing a person to accept an idea or belief, particularly the belief that the nurse can help and that the person will in time feel better.
Example: "Many other people have found it helpful to talk about this."

Technique: Reinforcement of behavior
Definition: Giving the patient positive responses to adaptive behavior.
Example: "That's the first time you were able to defend yourself with your boss, and it went very well. I'm so pleased that you were able to do it."

Technique: Support of defenses
Definition: Encouraging the use of healthy, adaptive defenses and discouraging those that are unhealthy or maladaptive.
Example: "Going for a bicycle ride when you were so angry."
Was very helpful because when you returned you and your wife was able to talk things through."

Technique: Raising self-esteem:
Definition: Helping the patient regain feelings of self-worth
Example: "You are a very strong person to be able to manage the family all this time. I think you will be able to handle this situation, too."

Technique: Exploration of solutions
Definition: Examining alternative ways of solving the immediate problem.
Example: "You seem to know many people in the computer
Field. Could you contact some of them to see whether they
Might know of available jobs?

Phases of Crisis Intervention (The Role of the Nurse):

Phase 1. Assessment
A nurse might perform some of the following assessments:

- Ask the individual to describe the event that precipitated this crisis.
- Determine when it occurred.
- Assess the individual's physical and mental status.
- Determine if the individual has experienced this stressor before. If so, what methods of coping were used? Have these methods been tried this time?
- If previous coping methods were tried, what was the result?
- If new coping methods were tried, what was the result?
- Assess suicide or homicide plan and means.

- Assess the adequacy of support systems.
- Determine the level of pre-crisis functioning.
- Assess the available support systems, and ability to solve problem.
- Assess the individual's perception of personal strengths and limitations.
- Assess the individual's use of substances.

Some nursing diagnoses that may be relevant include:
- Ineffective coping
- Anxiety (severe to panic)
- Disturbed thought process
- Risk for self- or other-directed violence
- Fear

Phase 2. Planning of therapeutic Intervention

❖ The nurse selects the appropriate nursing actions for the identified nursing diagnoses.

❖ In planning the interventions, the type of crisis, as well as the individual's strengths and available resources for support, are taken into consideration.

❖ Goals are established for crisis resolution and a return to the pre-crisis level of functioning.

Phase 3. Intervention: The following interventions are the focus of nursing in crisis intervention:

1) **Environmental manipulation:** By changing the patient's physical or interpersonal situation. These interventions may remove stress or provide situational support.
2) **General support:** The nurse uses warmth, acceptance, empathy, and reassurance to provide general support to the patient.
3) **Generic approach:** It is designed to reach high risk individual and large groups as quickly as possible.

It applies a specific method to all individuals faced with similar type of crisis (Social crisis).

4) **Individual approach:** It is similar to the diagnosis and treatment of specific problem in a specific patient.

It is useful in (situational and maturational crisis).

- Use a reality-oriented approach. The focus of the problem is on the here and now.

- Establish a rapid working relationship by showing acceptance by active listening.
- Promote an atmosphere for verbalization of true feelings.
- Set limits on aggressive, destructive behaviors.
- Clarify the problem that the individual is facing.
- Help the individual determine what he believes precipitated the crisis.
- Identify external support systems and new social networks in times of stress

Phase 4. Evaluation of Crisis Resolution

- Have positive behavioral changes occurred?
- Has the individual developed more adaptive coping strategies? Have they been effective?
- Does the individual believe that he could respond with healthy adaptation in future stressful situations?
- Can the individual describe a plan of action for dealing with stressors that precipitated this crisis?

Modalities of crisis intervention:

1) Mobile crisis program:
Its team provide crisis intervention to individuals, families, and communities. The nurse, who is a member of mobile crisis team, should be able to provide on-site assessment, treatment, referral, crisis management and educational services to patient, family and community at large.

2) Telephone contacts:
Crisis intervention is sometimes practiced by telephone rather than through face-to-face contact. The nurse should have effective listening skills to provide crisis intervention to victims.

3) Group work:
People who have common traits of stressors will form a group. The group provides an opportunity for members to express common concerns and experiences, foster hope and build mutual support.
The nurse and the group help the patient solve the problem &reinforce new problem-solving behavior.

4) Disaster response: Nurses are called on when social crisis occurred. They have important role in dealing with psychosocial problems of disaster victims. In this time the nurse go to places where victims are likely to gather such as hospitals.

5) Victim outreach program:
Using crisis intervention techniques to identify the needs of victims and then connect them with appropriate referral and other resources. Nurses work in this program immediately after the crisis.

6) Crisis intervention centers:

They provide emergency psychiatric care and counseling to victims, experiencing extreme stress or conflicts often involving suicide attempts. They also provide services 24 hours a day. The primary objective of them is to help the person cope with immediate problem and offer guidance & support for long term therapy.

7) Health education:

Nurses identify people who are at high risk for developing crisis and in teaching coping strategies to avoid development of crisis. Nurses are aware of available services and change public attitude. They also obtain information about how deal with potential crisis.